

Cognitive Representations of Self-Harm as Predictors of Coping Orientation and Attitudes Towards Service Engagement

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BACKGROUND

- We know that people understand their self harm in diverse and individual ways
- This aspect of lived experience is crucial, but how it affects other outcomes is unclear
- How we understand our health has been shown to be important to our general wellbeing, but this has not been explored in the context of self-harm.

METHODS

- Cross-sectional survey with **113** individuals with experience of self-harm.
- Outcomes of interest: coping styles and attitudes to accessing mental health services**
- An adapted version of the Brief Illness Perception Questionnaire was adapted to capture how people understand their own self-harm
- Multiple linear regression** was used to evaluate whether **understandings of self-harm have predictive power**
- Participants aged > 18 and based in the UK

WHAT ARE COGNITIVE REPRESENTATIONS & PERCEPTIONS?

- Perceptions** : Information that feeds into cognition
- Cognitive representations**: mental structures of knowledge and experience
- Conceptualisations**: our understandings of the world

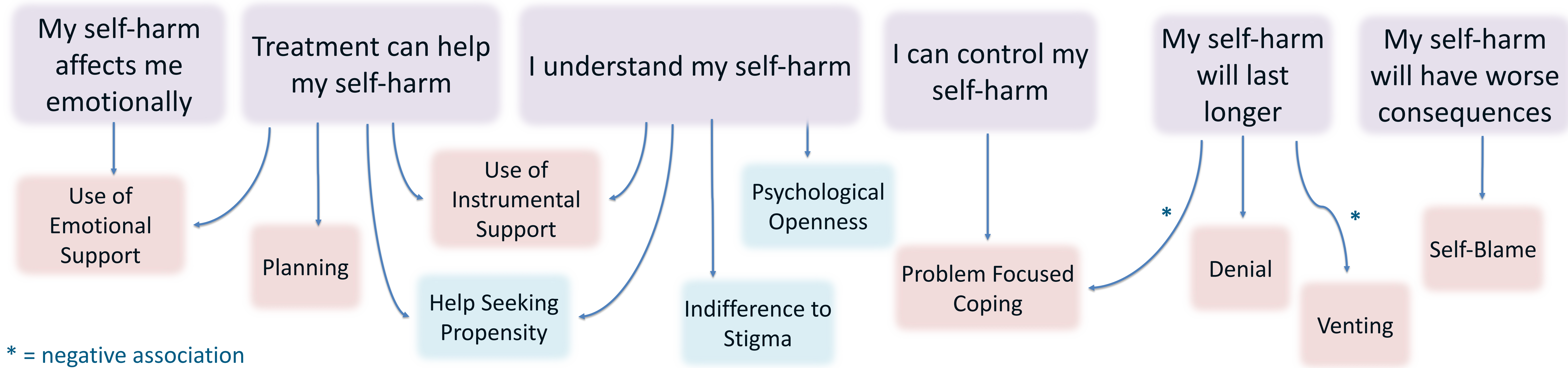
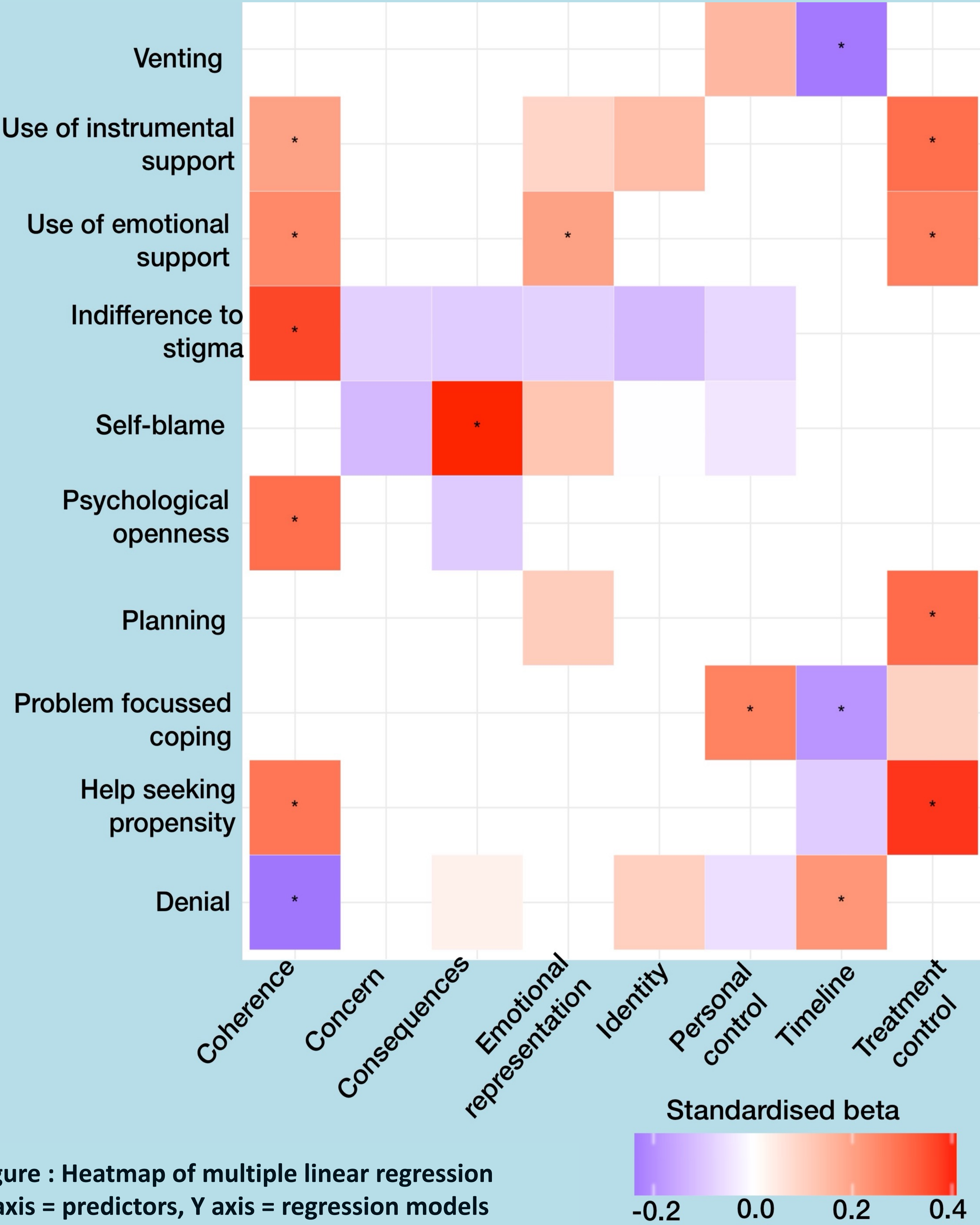
AIMS

- How do cognitive representations of self-harm predict coping styles and intentions to access services?
- What are the effects of how we understand self-harm?

RESULTS

- All regression models were significant (p-values 0.001 to 0.02, R² 0.051 to 0.189)
- All cognitive representations but identity and concern significantly predicted at least one outcome
- Greater perceived insight was the most positive cognitive representation, predicting greater help-seeking (p-values all <.01), adaptive coping outcomes, greater use of support, and less denial.
- Understanding self-harm to have worse consequences predicted higher self-blame (p < .01)
- Understanding self-harm to last longer predicted greater denial, reduced venting, reduced problem-focussed coping
- To account for missing data, a sensitivity analysis using multiple imputation via chained regressions was run. Density plots and imputed distributions were compared, and the results did not differ from the primary analysis

[See full results ->](#)



WHAT DO THESE FINDINGS MEAN?

- How people **understand their own self harm** is a **clinically important aspect of lived experience**
- Some ways of understanding self-harm **predicted negative coping styles** and **poorer attitudes towards accessing services**
- Other cognitive representations were predicted multiple positive outcomes- feeling that an individual understands their self-harm is the, most positive

LIMITATIONS

- Self-harm is a complex and diverse behaviour, this study did not collect nuanced data such as frequency or method of self harm, and no demographic data due for ethical reasons
- Participants were recruited from online forums and so not representative of all individuals who engage in self-harm.